

Session Objectives

THE STUDENT PHYSICIANS WILL BE ABLE TO:

1. Understand and become familiar with common psychological tests and identify their usefulness in patient populations.
2. Comprehend the differences between Objective and Projective tests
3. Comprehend the utility of Neuropsychological Examinations

Overview of Diagnostic Rating Scales and Psychiatric Instruments

Uses of Diagnostic Psychiatric Instruments:

- To help ensure the accuracy of a diagnosis (supplementing clinical interview, review of medical records and collateral sources) for clinical and research purposes
- To quantify the severity of symptoms
- To quantify the effectiveness, or lack thereof, of a given treatment modality

- **USES:**
- Diagnostic Interviews
- Helps increase reliability of psychiatric diagnoses (due to perceived unreliability; from different countries or cultural backgrounds)
- Example: Structured Clinical Interview for DSM-V (SCID) The most commonly used diagnostic instrument in psychiatry

- **Structured Clinical Interview for DSM-V (SCID)**
- A semi-structured interview that applies DSM-V criteria to a patient that covers most of the major disorders
- Administered by a clinician and may take upwards of 2 hours to complete
- Other example: Mini-International Neuropsychiatric Interview (MINI) Administered by a clinician and takes only 15-30 minutes; yes/no

Depression Scales

Example: Hamilton Rating Scale for Depression

- *(HAM-D) Aims to quantify the severity of depression in patients who already have a diagnosis of major depression
- *Administered by a clinician and takes approximately 20 minutes-used to create tx plans
- *A useful tool for measuring the progress of a patient during the course of treatment, in either research or the clinical setting
- *Is the most widely studied instrument for depression, and its reliability and validity are high
- *Scores on HAM-D-17: 0-7 (not depressed), 7-15 (mildly depressed), 15-25 (moderately depressed), over 25 (severely depressed)

Example: Beck Depression Inventory (BDI);

The BDI-II is a self-report measure comprised of 21 items reflecting specific cognitive, affective, and physical symptoms of depression. Scores range from 0 to 3, with higher numbers indicating greater symptom severity. If more than one statement from a given item is chosen by the patient, the statement of greatest severity is scored. The maximum total score is 63

- * Most widely used to assess depressive symptoms in adolescents and adults

- * Self-administered; 5-10 minutes

- * May be used as a screening tool or to determine the degree of improvement over time

Psychosis Scales

- **Brief Psychiatric Rating Scale** (BPRS) Covers symptoms of psychosis, but also addresses depression, and anxiety symptoms-18 symptom constructs
- -useful in gauging efficacy of treatment in pts with psychoses

* **Positive and Negative Symptoms Scale**
(PANSS) Frequently used in trials of antipsychotic drugs

Anxiety Scales

- Hamilton Rating Scale for Anxiety (HAM-A) The most widely used scale for measurement of anxiety
- Yale-Brown Obsessive Compulsive Scale (Y-BOCS) The most widely used scale for assessment of severity of obsessive-compulsive disorder symptoms
- GAD-7 and PHQ-9-used in ECW at NYITCOM-brief screenings of anxiety and depression

Cognitive Impairment Scales

Overview:

- *Are useful as an initial screen for organic psychopathology
- *Results of these scales can help the clinician decide whether or not to request formal neuropsychological testing or imaging studies
- *May be influenced by the patient's intelligence, level of education, and literacy

Cognitive Impairment Scales

Examples: *Folstein Mini-Mental State Examination
(MMSE)

- *The most widely used instrument for measuring cognitive impairment

- *May be administered by non-clinicians; takes less than 10 minutes

Montreal Cognitive Assessment (MoCA)

*The Montreal Cognitive Assessment (MoCA) was designed as a rapid screening instrument for detecting Mild Cognitive Impairment (MCI)-55-85y/o. It assesses different cognitive domains: attention and concentration, executive functions, memory, language, visuoconstructional skills, conceptual thinking calculations, and orientation. Time to administer the MoCA is approximately 10 minutes. The total possible score is 30 points; a score of 26 or above is considered normal.

Intelligence Tests

IQ Testing Example: Weschler series (preschool, children, adults) Provides three major IQ test scores: the Full Scale IQ, the Verbal IQ (vocabulary, similarities, arithmetic, digit span, information, and comprehension), and the Performance IQ (picture completion, digit symbol, block design, matrix reasoning, and picture arrangement)

All have mean of 100

Full-Scale IQ >130-very superior, 120-129-superior, 110-119-high average, 90-109-average, 80-89-low average, 70-79-borderline, <69-mentally retarded

Tests of Personality, Psychopathology and Psychological Functioning

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Objective Tests: Also known as self-report tests, are designed to clarify and quantify a patient's general personality functioning and psychopathology

Example: Minnesota Multiphasic Personality Inventory-2 (MMPI-2)

- A 567-item true/false self-report test of psychological functioning

- Contains ten clinical scales that assess major categories of psychopathology

- Four scales of validity are used to assess for the accuracy and truthfulness of the test-takers answers

Similar: Millon-Clinical Multiaxial Inventory-III (MCMI-III)

Tests of Personality, Psychopathology and Psychological Functioning

Projective Tests:

- Differ from objective tests in that they are less structured and require more effort on the part of the patient to make sense of and to respond to, the test stimuli
- Provide insights into the patient's typical style of perceiving, organizing, and responding to ambiguous external and internal stimuli

Projective Tests:

Example: Rorschach Inkblot Test

- test of whole personality functioning
- Consists of 10 cards of symmetrical inkblots; “What might this be?”
- Particularly useful for quantifying a patient’s reality contact and the quality of their thinking
- Requires the subject to interpret the questions by projecting his or her emotions, conflicts and defense mechanisms into the answers
- Examiner must have special training in interpretation of subject’s responses
- Most commonly used projective personality test

Example: **Thematic Apperception Test (TAT)**

- Useful in revealing a patient's dominant motivations, emotions, and core personality conflicts
- Consists of a series of 20 cards depicting people in various interpersonal interactions; ambiguous scenes; "Make up a story around this picture"

Example: **Sentence Completion Test**

- Used to identify a patient's worries and motivations by verbal associations
- Believed to reveal a patient's underlying thoughts or feelings
- Patient is given a sentence stem and then they complete the rest of the sentence

Neuropsychological Assessment

Overview:

The main goal of a neuropsychological evaluation is to:

- Better understand the relationship between your brain and behavior, mood and thinking (cognitions).

- Such tests can help with Diagnoses, pinpoint cognitive strengths and deficits, aid with treatment planning.

Examples of Neuropsychological Assessments:

- Halstead-Reitan battery of neuropsychological tests
- Brief Neuropsychological Test example:
Dementia Rating Scale (DRS)
- Repeatable Battery of Adult Neuropsychological
Status (RBANS)

Repeatable Battery for Neuropsychological Status (RBANS)

- Brief neuropsychological screening battery widely used to assess cognitive function in dementia, head injury.
- used in 20-89 y/o, research protocols-easy/quick, alternate forms
- 12 Subtests: Immediate and Delayed Memory, Attention, Visuospatial/Construction, Language
- Index Scores: Very Superior, Superior, High Average, Average, Low Average, Borderline & Extremely Low

Summary Slide

- Understand common psychological tests and identify their usefulness in patient populations
- Be knowledgeable about common neuropsychological tests and their utility in patient populations